جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : رسميه عبدالشافي عبدالشافي الخمري

المحافظة : المنوفيه **الحي / المدينة :** اخري **العنوان :** 4ش كوم امبو مصر الجديدة

رقم البطاقة : 24106201701264 **رقم التليفون :** 01001517530 **تاريخ تسجيل الإستمارة :** 19-10-2026

العمر(تاريخ الميلاد) : 20-06-1941 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع:(----------)

المقر بما فيه
التوقيع:(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي الفقرات القطنيه بالصبغه [12771] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: رسميه عبدالشافي عبدالشافي الخمري
National ID: 24106201701264

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the lumbar spine

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


رسمية
عبدالشافى عبدالشافى الخمرى
١ ش كوم امبو
مصر الجديده - القاهره
٢٤١٠٦٢٠١٧٠١٢٦٤
١٩٤١/٠٦/٢٠
I65376425

٢٤١٠٩٢٢٠١٧٠١٢٦٤ ٢٠١٩/١٢

بدون عمل

انثى مسلمة ارملة

البطاقة سارية حتى ٢٠٢٦/١٢/٠٤

١

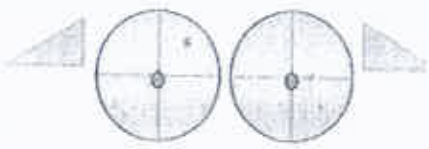
Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : سعيه عبدالشافى عبدالشافى الضرى Sex:
2-ID number:

3- Medical number: 83230175
Female
Old Med : 3024-2024
Patient Age : 84 Y



Family history		Phone	01001517533	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	13/01/2025
Co morbidity	IHD STENT	Menopausal status	POSTMENOPAUSAL	Initial STAGE			

Clinical data

Breast Imaging :
rt breast malignant looking mass lesion , ipsilateral axillary pathological LN BIRADS 5
lt breast UOQ gps microcalcifications BIRADS 3

Mets Workup :
pet ct: metabolically active rt breast neoplastic mass with ipsilateral axillary & mediastinal nodal deposits
metabolically active 4th dorsal vertebra mixed lytic sclerotic osseous deposits
bone scan: 2 equivocal bone lesions

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	5/8	0		

T	N	M
cT4b localized	1	1bone

Sonomammography

PATIENT HISTORY
RT PAROTID MASS, BIOPSY: malignant, started 1st line FEMARA x2months
Surgery done for parotid and breast mass (invading skin).
rt breast mass lumpectomy:IDC GII, free skin, margin
parotidectomy mets adenocarcinoma
ER 8/8 PR 0/8 HER2 0

MDT OUTCOME & PLAN

In view of being metastatic, continue on 1st line FEMARA + Zometa every 3 months

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE MDT FORM]]

Patient Name	: الاسكندر / رسميه عبد الشافي عبد الشافي		
Patient No.	: 2410602017012 64	Request No.	: 13008641
Path No.	: SR-1286/24	Request Date	: 08/09/2024
Sex	: Female	Verification Date	: 18/09/2024
Age	: 83 Y	Financial Category	: تأمين دار السلام - تغطية الدولة
Referreing Doc :		Department	: Out Patient
		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass & Right parotid FNAC.
Nature of Specimen: Slide Revision.

Gross Examination.
- 3 paraffin blocks labeled by (200706/24) and 5 slides were received.
- Two smears & One cell block received .

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* DCIS: No.
* Angioinvasion: No.
* Perineural invasion: No.
* IPT:
- ER: Strong nuclear reaction in 90% of tumor cells (score 8 /8).
- PR: Strong nuclear reaction in 7% of tumor cells (score 5 /8).
- Her2: No membrane reaction in the tumor cells (score 0).

* Examination of the received smears and cell block prepared revealed Hypocellular hemorrhagic smears with few groups of epithelioid cells.

Diagnosis: (Revised)
- Right breast mass, Slide revision:
* Invasive duct carcinoma G II.
IPT:
- ER: Positive (score 8 /8).
- PR: Positive (score 5 /8).
- Her2: Negative (score 0).

Verified by :
DR.Menna Nabil

غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE PATHOLOGY REPORT]]

Patient Name	: الأستاذة / رسميه عبد الشافي عبد الشافي	Request No.	: 13008641
Patient No.	: 2410602017012 64	Request Date	: 08/09/2024
Path No.	: SR-1286/24	Verification Date	: 18/09/2024
Sex	: Female	Financial Category	: برنامج رئاسي للنساء - تغطية الدولة
Age	: 83 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

- Parotid Gland, FNAC (Slide revision):
Hemorrhagic smears with few groups of epithelioid cells.
Excision is recommended for proper assessment.

Signature
Prof. Dr. Lubna Omar
Dr. Toqa Medhat.

Dr. Marwa Mahmoud

Verified by :
DR.Menna Nabil

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE PATHOLOGY REPORT]]

Patient Name	: الأستاذه / رسميه عبد الشافى عبد الشافى الخمرى	Request No.	: 13004600
Patient No.	: 24106201701264	Request Date	: 11/12/2024
Path No.	: 4957/24	Verification Date	: 02/01/2025
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 83 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass
Nature of specimen:
Lumbectomy & parotidectomy , collected at 10/12/2024, 10:30 am

Gross examination
Two specimens wrere received:
1) Breast :Fibrofatty tissue piece measured 9x8x3 cm , covering skin measured 5.5x1.5 cm, serialling revealed grey white mass measured 3.5x2.5x2.5 cm
2)Parotid :Fibrofatty tissue piece measured 3.5x2.5x1 cm , covered by skin 3.5x1.8 cm , serialling revealed firm grey white mass 0.9x0.9 cm , with well defined firm grey white nodule 0.3x0.3 cm with 0.1 cm, in between
-The mass is away 0.1 cm from both skin and deep margin , inked all around , multisected
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
1)Breast mass :
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II
* DCIS: No
* Angioinvasion: Couldn't be detected in the received biopsy
*Perineural invasion: No
*Margins :Free
*Skin :Free
2)Parotidectomy ,
-Both nodules showed Metasatic adenocarcinoma mostly of breast origin with unrematkable surrounding salivary tissue

Verified by :

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Pathology Report
[[DOCTYPE PATHOLOGY REPORT]]

الأستاذه / رسميه عبد الشافى عبد الشبر		Request No.	: 13004600
06201701264		Request Date	: 11/12/2024
4957/24		Verification Date	: 02/01/2025
: Female		Financial Category	: اورام دار السلام - تغطة الدولة
: 83 Y		Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

• IPT (Positive control showed appropriate reactivity) :
*GATA3: Positive
* ER: Strong nuclear reaction in 80 % tumor cells (Allred score 8/8)
* PR: No nuclear reaction in tumor cells (Allred score 0/8)
* HER2: No membrane reaction in of tumor cells (score 0)

Diagnosis
Right breast mass, lumbectomy:
Invasive duct carcinoma, Grade II
-Free skin , free margins

-Parotidectomy, (face mass):
-Metastatic adenocarcinoma mostly of breast origin
IPT:
* ER: Positive (score 8/8).
* PR: Negative (score 0/8).
* HER2: Negative (score 0).

Signature
Dr. Rana Fathy
Dr.Heba Hassan

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE PATHOLOGY REPORT]]

PATIENT NAME: RASHA ABDUSSHAFIE ABDUSSHAFIE EL ISHAMRY
GENDER: Female
DATE : 07-Aug-2024 09:27 AM

PATIENT ID: 2022401071A
AGE: 83 y
ORDER: Z105051,Z105052

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULYRASOUND

INDICATION:
- Right breast lump

TECHNIQUE:
- CC and MLO view.
- Complementary ultrasound

FINDING:
- Scattered fibroglandular densities (ACR B)
- Right breast LOQ deeply seated mass lesion
- Left UOQ few grouped microcalcifications the largest measures 6 x 6 mm and 89 mm away from the nipple (Arrows).
- Bilateral widely scattered microcalcific foci are noted
- Complementary Ultrasound showed right breast 7 o'clock irregular spiculated hypoechoic vascular mass lesion measures 28 x 32 mm and seen invading the overlying skin and anterior ductal extension measures 33 mm
- No left solic or cystic mass lesions.
- Normal left skin thickness and contour.
- Normal nipple areola complexes
- Nonspecific left axillary lymph nodes.
- Right axillary level I few pathological lymph nodes the largest one measures 18 x 15 mm

OPINION:
- Right breast malignant looking mass lesion and ipsilateral axillary pathological lymph nodes (BIRADS5) for biopsy.
- Left breast UOQ grouped microcalcifications (BIRADS3) for stereotactic biopsy.

RECOMMENDATION:
Referral to clinic

Reported By:

Dr. WAFAA ALI HEIDER,Msc 07-Aug-2024 09:09 AM

Mammogram
[[DOCTYPE MAMMOGRAM]]

PATIENT NAME: RASMA ABDUSSHAFIE ABDUSSHAFIE EL KHAMRY
GENDER: Female
DATE: 23-Aug-2024 11:52:40

PATIENT ID: 2022/010714
AGE: 83 y
ORDER: 2998837

Whole body [18F] FDG PET-CT scan

CLINICAL HISTORY:

83 years old female patient, presented with right breast mass cytologically proven invasive duct carcinoma and right parotid gland pathology proven neoplasm of uncertain malignant potential.

Reason for the study: detection of FDG avid neoplastic lesions.

TECHNIQUE:

- Radiopharmaceutical: 18F FDG, Intravenously.
- Time from 18F-FDG injection to scan:60 min
- PET/CT images were acquired from the skull through the upper thighs & CT images were acquired for the purpose of anatomical localization and attenuation correction. Images were reviewed in the transaxial, coronal, and Sagittal planes. CT without contrast was performed.
- Patient's weight: 78Kg - Patient's height: 157cm -Bl. Gluc. Level: 111mg/dL
- Additional notes: SUV max normalized to body weight was used.
- Reference hepatic activity (SUV max):-2.8.

FINDINGS:

The breasts and axillary regions:

- FDG avid right breast lower quadrant deep seated skin infiltrating neoplastic mass is seen and closely related to chest wall muscles measuring 3x2.5 cm with SUVmax~5.5.
- FDG avid right axillary (level I) measuring 2.4x1.9cm cm with SUVmax~4.1. Low grade metabolic activity by other level I and level II lymph nodes are seen.
- No FDG avid lesions within the left breast or the axillary lymph nodes.

Head and Neck:

- FDG avid right parotid gland superficial lobe lower aspect nodule seen measuring 1.3x1.2cm with SUVmax~6.2.
- Age related brain involutional changes with no hypermetabolic brain lesions.
- physiologic FDG uptake is seen in the nasopharynx, oropharynx, and larynx.
- physiologic FDG uptake is seen in submandibular and left parotid salivary

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مستشفى بهية الشيخ زايد
Mammogram
[[DOCTYPE MAMMOGRAM]]

Chest:
- FDG avid prevascular and left hilar lymph nodes are seen, the former is the largest measuring 19x12 mm with SUVmax~4.1. Other low grade prevascular and retrocaval lymph nodes are also noted.
- No FDG avid lung nodules or masses detected.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart.

Abdomen and pelvis:
- Physiologic diffuse FDG uptake is seen over average sized liver. No FDG avid focal lesions detected.
- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal appearance of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- Normal non-contrast CT appearance of the pancreas, and both supra-renal with no FDG avid lesions detected.
- No evidence of ascites or abdominal collections noted.
- Diffuse FDG activity is seen over lower anterior wall subcutaneous collection associated with smudging of fat planes and surgical clips showing SUVmax~1.3 : for clinical correlation.
- Prominent both adnexae showing low grade FDG uptake: for sonographic correlation

Musculoskeletal:
- FDG avid heterogeneous mixed sclerotic and lytic osseous lesion seen implicating 4th dorsal vertebra with SUVmax~4.9.
- Other multiple low grade FDG avid bone marrow based purely sclerotic osseous lesion involving the body of the sternum, few dorso lumbar vertebrae, bilateral ribs rib, both iliac bones, sacrum and left femoral trochanter are also noted :likely metastatic in nature.

Conclusion:
- FDG PET/CT study for a known case of right breast cancer revealed;
  - Metabolically active right breast neoplastic mass with ipsilateral axillary and mediastinal nodal deposits.
  - Metabolically active 4th dorsal vertebra mixed lytic and sclerotic osseous deposit as well as low grade metabolically active wide spread bone marrow based purely sclerotic osseous lesions :

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مستشفى بهية الشيخ زايد
Mammogram
[[DOCTYPE MAMMOGRAM]]

Reported By:
Dr. Meryem Aly, MSc
Dr. Mahmoud Ibrahim, MD
Dr. ElShaymaa Hussein, Nuclear Medicine Consultant 27-Aug-2024 03:13 PM

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مركز بهية للأورام
٤ شارع علوية أول شارع الدرج بجوار نادي الجزيرة
فستا في بهية الشيخ زايد
٦ المحور المركزي، الشيخ زايد أمام كلية الهندسة
Mammogram
[[DOCTYPE MAMMOGRAM]]